

Winning the AI-search war for VA disability claims

MilitaryDisabilityNexus.com (MDN) can realistically become a top-cited source for AI search engines on VA disability claim queries within 6-12 months — but **not by competing head-on with VA.gov, CCK Law, or Hill & Ponton on "how to file" content**. The site's legitimate advantage is clinician-authored, regulatorily precise, YMYL-grade **medical-evidence content** — exactly the query space AI engines trained on the NPR/War Horse VA-warning-letter stories are hungry to cite. The gap to close is large: MDN has only ~5 blog posts, 3 case studies, no named clinician bylines, and zero content on PACT Act, TDIU, SMC, appeals, MST, TBI, Gulf War Illness, tinnitus, migraines, or primary PTSD — all of which represent high-volume AI-search queries.

Three findings reshape the strategy. First, Google's September 11, 2025 Quality Rater update pulled VA-claim content into **three simultaneous YMYL categories** (Health + Financial Security + Government/Civics) — the highest scrutiny tier in existence, which makes named-clinician E-E-A-T non-optional. Second, **Section 1151 (VA medical malpractice) is a nearly empty SERP** — only 6-8 authoritative pages rank for most variants, and MDN already has a service page that can anchor a category-leading pillar. Third, **Perplexity draws 46.7% of top-10 citations from Reddit and 31% from social/community sources** — meaning MDN's dormant (/community) forum is either the site's largest untapped asset or a liability that should be removed. The report below translates these findings into a prioritized content, keyword, and GEO/AEO roadmap.

1. Executive summary

The competitive reality. The VA disability SERP is a three-tier ecosystem. Tier 1 (VA.gov, eCFR, Reddit, Wikipedia) is undisplaceable — plan to be cited *alongside* these sources, not instead of them. Tier 2 (CCK Law, Hill & Ponton, Woods & Woods, Berry Law, VA Claims Insider) dominates appeals/TDIU/ratings content through sheer content volume and attorney bylines. Tier 3 (Trajector Medical, Telemedica, REE Medical, Valor 4 Vet, Prestige Veteran) is MDN's true competitive set — most Tier 3 peers have thin bylines, no clinician schema, and limited regulatory citations. **MDN can win Tier 3 outright in 6-12 months** and steal long-tail share from Tier 2.

The four strategic pivots.

1. **Stay in the medical-evidence lane.** Never write "how to file" in imperative voice (that's the trigger for 38 USC § 5901/5904 accreditation issues (The Texas Tribune) and the VA warning letters sent to Trajector, VA Claims Insider, and Veterans Guardian). (The War Horse) Instead write "What medical evidence the VA evaluates for [condition]."
2. **Publish named clinicians with full credentials and Physician schema.** This is MDN's single largest E-E-A-T gap and single largest competitive advantage waiting to be unlocked.
3. **Own the 1151, secondary-condition-chain, and DBQ long-tail clusters.** These are nearly empty SERPs with direct commercial alignment.

4. **Activate — or remove — the community forum.** A functioning branded Q&A earns Perplexity/ChatGPT citations; an empty page is an E-E-A-T liability.

The 90-day priority stack. Deploy Organization + Physician + MedicalWebPage + FAQPage schema sitewide; publish named clinician bios; build 15 foundational condition/topic articles in the AI-optimized template; launch the 1151 pillar hub; activate the community with 25 seeded clinician-answered threads.

2. Top 27 veteran pain points (ranked by frequency and content-opportunity value)

These are the complaints that dominate r/VeteransBenefits, Quora, Veterans Law Blog comments, law-firm client correspondence, and congressional oversight records. Each represents a content opportunity — but the highest-ROI ones are those where **few good resources currently exist online** (flagged "GAP").

1. **"Drive-thru" / rushed C&P exams.** Exams under 20 minutes, examiners not looking up from the computer, (VMHA) no goniometer, no flare-up testing. Senator Warren's office has documented exams conducted in broom closets and hotel rooms. (Prestige Veteran Medical C... (Stars and Stripes) The representative sentiment: *"The examiner spent 10 minutes with me and refused to discuss my depression symptoms."* (VMHA) Affects PTSD, MDD, musculoskeletal, TBI claims most severely.

2. **Unqualified or wrong-specialty C&P examiners.** Mental-health exams conducted by nurse practitioners, orthopedic exams by audiologists, examiners who clearly did not review the C-file. (Prestige Veteran Medical C... CCK Law documents: *"An audiologist or someone that might test hearing levels should not be evaluating you for an orthopedic condition."* (CCK Law +2)

3. **"Pencil-whipped" DBQs with fabricated ROM findings.** Examiners mark "full range of motion" without ever conducting the test. (VA Claims Insider) (Veterans Benefits Network) Ignored Correia and Sharp flare-up requirements. (JVDS) (CCK Law) Most acute in back, knee, shoulder, and cervical claims.

4. **Examiner negative nexus with no rationale or misquoted literature.** The examiner writes "less likely than not" with circular reasoning, (Veterans Disability Info) or cites a study whose actual conclusion supports the veteran. (JVDS) A very citable gap for a clinician-authored rebuttal-letter guide.

5. **Sleep apnea secondary to PTSD denied despite strong literature.** Roughly 69% of OIF/OEF PTSD veterans are at high OSA risk per peer-reviewed research, (CCK Law) (VA Claims Insider) yet secondary OSA denials are endemic. Examiners say "sleep apnea is a physical condition not caused by PTSD" without addressing aggravation. (VA.gov) Highest-ROI secondary-chain topic on the internet.

6. **Tinnitus capped at 10% under DC 6260 — and the pending rule change.** Proposed VASRD revisions would eliminate standalone tinnitus as a separately rated condition, (VA Claims Insider +8) a 2025-2026 story with massive search demand and almost no clinician-authored explanation.

7. Low-ball PTSD ratings at 30% or 50%. Veterans with genuine 70% occupational/social impairment ([HadIt.com Veterans](#)) repeatedly rated at 30-50% because the C&P captured a "good day" or examiner didn't probe night terrors, avoidance, or social withdrawal.

8. DAV/VFW/American Legion representation treated as useless. Veteran sentiment from Veterans Law Blog comments: *"The DAV did as little as possible representing my claim. Once I was denied, they dropped me like a hot potato."* ([Veteranslawblog +2](#)) Another: *"Most VSOs I've met don't know what 38 CFR is, let alone 38 CFR 4.16(b)."* ([VA News](#)) Some veterans report good experiences, but complaints dominate.

9. Nexus letter confusion and cost shock. Veterans don't know what a nexus letter is, whether they need one, whether treating doctors can write them, or why market pricing ranges from \$500 to \$3,000+. ([Homefrontgroup](#)) **GAP:** transparent pricing explanations and "what makes a strong nexus letter" breakdowns are rare.

10. PACT Act confusion — still denied post-PACT. Veterans think PACT Act means automatic approval; they still need diagnosis plus qualifying service location and period. Previously-denied claims require supplemental filing, ([Hill & Ponton +4](#)) and the VA's promised proactive re-review is inconsistent. ([Veterans Disability Info](#))

11. TDIU eligibility misunderstandings. Veterans don't know they qualify; don't know about extraschedular TDIU under **38 CFR 4.16(b)**; ([VA News +3](#)) fear TDIU bars them from any work; confused about "substantially gainful" versus marginal employment versus sheltered employment; ([Hadit](#)) don't realize passive income (rent, investments) doesn't count. ([Woodslawyers](#)) ([Veterans Benefits Knowled...](#)) **GAP.**

12. Appeal lane confusion — HLR vs. Supplemental vs. BVA. HLR allows no new evidence; ([Disability Law Group +4](#)) vets submit it anyway and it isn't considered. The one-year deadline ([VA](#)) trap forfeits original effective dates. ([Hill & Ponton](#)) BVA's three docket options ([VA](#)) have wildly different timelines. ([VA Claims Insider](#))

13. Effective date disputes and lost back pay. VA uses claim-receipt date instead of day-after-discharge when filed within one year of separation; ignores intent-to-file filings; misapplies liberalizing-law (Nehmer, PACT) dates. Veterans lose years of retroactive payments. **GAP** on Nehmer/PACT retroactivity walkthroughs.

14. VA.gov and eBenefits portal dysfunction. Status tracker stuck on "Preparation for Decision" for months then jumps without explanation; decision letter arrives after the deposit hits the bank; ID.me vs. Login.gov migration caused login chaos.

15. Claim deferrals — poorly explained. Partial rating plus deferral on other conditions, typically 30-90 days ([VA Claims Insider](#)) but stretching to 4-6 months. Not appealable. VA OIG found VBA issued 23,200 unwarranted deferrals costing ~\$8.8M. **GAP.**

16. Backlog and wait-time anxiety. 146.4 days average completion; 249,159+ claims backlogged (over 125 days) as of January 2025. Financial stress during the wait is the top emotional pain point.

17. Rating reduction fears preventing increase claims. Veterans terrified that filing an increase triggers review of existing ratings, (Hadit) or that one good C&P day causes reduction. Confusion about the 5-year stabilization rule, 10-year rule (service connection), 20-year rule (rating), and P&T protection. **GAP.**

18. Buddy statement confusion. Veterans don't know the correct form (21-10210 vs. 21-4138), who qualifies, what to include, or how to find lost contacts — especially fatal for PTSD stressor and MST claims.

19. C-file and STR access frustrations. VA Form 20-10206 FOIA/Privacy Act requests take months; (VA) pre-digital STRs routinely missing. **GAP** on mechanics of requesting and reading the C-file.

20. MST claim evidence burden. Veterans need "markers" (STR behavioral changes, transfers, pregnancy/STI tests). (ProVet Legal) (Military.com) Re-traumatizing C&P exams common. **GAP** on specifically what personnel-file entries count as markers.

21. Section 1151 claims opaque and rarely granted. Technical complexity of "additional disability from VA treatment," few VSOs know how to file, nearly zero veteran-facing guidance. **Highest-ROI GAP** — and MDN's direct service alignment.

22. Aid & Attendance complexity for elderly vets and surviving spouses. Net-worth limit, medical-expense deductions, three-year asset look-back (October 2018 rules), Housebound vs. A&A rate confusion. Predatory non-lawyer "pension planners" common.

23. PCAFC Caregiver Program denials and tier reductions. MISSION Act re-assessments bumped legacy caregivers off the program; the "unable to self-sustain in community" standard is opaque. **GAP.**

24. CUE claims misunderstood and misfiled. Veterans file CUE when HLR or supplemental would be appropriate. The undebatable-error standard is rarely explained in plain language. **GAP** on CUE motion template guidance.

25. Paid services (VCI, Trajector, Veterans Guardian) vs. free VSOs tension. Fees often 5× monthly rating increase. (HadIt.com Veterans +2) Non-accredited representation (NPR) is illegal under 38 USC § 5901/5904; (Congressman Chris Pappa...) companies skirt it by calling services "coaching." (Evrin Aği +2) This is precisely the positioning question MDN must navigate with scrupulous scope discipline.

26. SMC underutilization. Veterans don't know SMC-K (\$139.87 for loss of use of creative organ), (VA) SMC-S (housebound), SMC-L through T, or the bilateral factor. VA rarely grants SMC without explicit request. **GAP.**

27. Camp Lejeune — VA claim vs. Justice Act lawsuit. Two separate tracks with different rules and deadlines create overwhelm. (Ward Black Law) (Camplejeuneclaimscenter) Veterans unsure whether filing one waives the other.

Common veteran terminology to use verbatim in content (essential for AI semantic matching): C&P, IMO, DBQ, "at least as likely as not," "pencil-whipped," "drive-thru," MFR, ACE chart

review, QTC/LHI/VES, P&T, TDIU, "back door to 100%," VA math, schedular vs. extraschedular, HLR/Supp/BVA, ITF, retro, C-file, STRs, VBMS, 21-4138, 21-10210, DRO, RVSR, CUE, PFD, stressor letter, markers, Nehmer, bilateral factor, SMC-K/L/R, MUCMI.

3. Top 153 high-value keywords across 11 clusters

Volumes and difficulty are directional estimates (paid tool verification recommended). Priorities reflect both ranking feasibility and AI-citation potential.

A. High-volume informational head terms (22)

VA disability rating; VA disability calculator; VA disability pay chart 2026; VA disability rates 2026; VA disability claim; how to file a VA disability claim; VA disability benefits; VA claim process steps; VA combined rating calculator; VA disability rating chart; how to get 100 VA disability; VA disability back pay; effective date VA claim; VA intent to file; VA Form 21-526EZ; VA disability claim status; how long does a VA claim take; average VA disability rating; VA disability for veterans; VA service connection; types of VA disability claims; VA disability eligibility. (Most head terms are **High difficulty** — build for topical authority, not traffic wins.)

B. Long-tail question keywords — AI search goldmine (18)

How do I file a VA disability claim for sleep apnea; why was my VA claim denied; what is a nexus letter; how long does a VA claim take in 2026; what is the VA rating for PTSD; how do I increase my VA disability rating; can I work with 100% VA disability; what is the difference between TDIU and 100 schedular; how to prove a service connection; what happens at a C&P exam; how to get a nexus letter; what is the highest VA rating for migraines; can sleep apnea be secondary to PTSD; how much does VA disability pay in 2026; what does 38 CFR 4 mean; how do I appeal a VA decision; what is VA math; how do I file a supplemental claim. These cluster questions all start with interrogatives — the exact pattern AI engines extract as direct answers.

C. Condition-specific (32)

VA rating for sleep apnea; sleep apnea secondary to PTSD; sleep apnea secondary to sinusitis; sleep apnea secondary to rhinitis; VA rating for PTSD; PTSD secondary conditions; VA rating for tinnitus; tinnitus secondary conditions; VA rating for TBI; VA rating for migraines; migraines secondary to tinnitus; migraines secondary to PTSD; VA rating for back pain; VA rating for lumbar strain; VA rating for knee pain; VA rating for hypertension; hypertension secondary to PTSD; VA rating for diabetes; VA rating for IBS; VA rating for GERD; GERD secondary to PTSD; VA rating for radiculopathy; radiculopathy secondary to back pain; VA rating for depression; VA rating for anxiety; depression secondary to chronic pain; VA disability for MST; MST PTSD VA claim; VA rating for hearing loss; VA rating for erectile dysfunction; VA rating for asthma; VA rating for chronic sinusitis.

D. Appeal and denial (12)

VA claim denied what to do; Higher Level Review VA; Supplemental Claim VA; Board of

Veterans Appeals; CUE claim VA (**Low difficulty**); VA appeal wait time; VA Notice of Disagreement; how to appeal VA disability denial; VA Form 10182; VA Form 20-0995; new and relevant evidence VA; Court of Appeals for Veterans Claims.

E. TDIU/SMC/A&A (10)

TDIU requirements; TDIU vs 100 schedular; SMC VA; Aid and Attendance eligibility; 100 P&T VA; SMC-S housebound; SMC K rate 2026; how to apply for TDIU; SMC-R1 R2 Aid and Attendance (**Low difficulty**); CHAMPVA eligibility 100 P&T.

F. PACT Act (9)

PACT Act presumptive conditions; burn pit VA claim; PACT Act eligibility; Camp Lejeune VA disability; Agent Orange presumptive conditions; Gulf War illness VA claim; 2025 PACT Act additions bladder cancer (NVLSP) (**Low difficulty**); toxic exposure screening VA; hypertension Agent Orange presumptive.

G. Section 1151 — highest-opportunity cluster (7)

Section 1151 claim (**Low difficulty**); VA medical malpractice claim; 1151 vs FTCA (**Low**); how to file a Section 1151 claim (**Low**); 38 USC 1151 benefits (**Low**); VA negligence compensation (**Low**); 1151 claim evidence requirements (**Very Low**). Build a 3,000+ word pillar plus 5–7 spokes to dominate this cluster.

H. Nexus letter and medical opinion (10)

Nexus letter cost; how to get a nexus letter; IMO independent medical opinion VA; DBQ forms VA; nexus letter for sleep apnea; nexus letter for secondary condition; nexus letter template; what makes a strong nexus letter; private medical opinion VA claim; DBQ doctor near me.

I. Emerging/trending (6)

2026 VA rating schedule changes; new sleep apnea rating rules; new tinnitus rating criteria; VA mental health rating changes 2026; AI and VA claims (**Very Low** — first-mover advantage); PACT Act 2026 new conditions.

J. Transactional/commercial (7)

Nexus letter service; VA claim help; DBQ doctor; VA disability attorney (**High** — mostly law-firm territory); VA medical documentation service (**Low** — **brand-defensible exact match**); independent medical opinion for veterans; medical evidence for VA claim.

K. Secondary-condition chains — AI citation gold (10)

Erectile dysfunction secondary to PTSD (SMC-K qualifying); (VA.gov) insomnia secondary to tinnitus; heart disease secondary to sleep apnea; hypertension secondary to sleep apnea; kidney disease secondary to hypertension; IBS secondary to PTSD; depression secondary to tinnitus; anxiety secondary to tinnitus; sleep apnea secondary to asthma; migraines secondary to TBI.

The 10 highest-priority keyword targets for MDN: Section 1151 claim; 1151 vs FTCA; nexus letter cost; sleep apnea secondary to sinusitis/rhinitis; migraines secondary to tinnitus; VA medical documentation service; DBQ for [condition] cluster; new sleep apnea/tinnitus rating rules 2026; radiculopathy secondary to back pain; PACT Act 2026 new conditions.

4. Website audit — what MDN has, what it's missing

The entity. MAGLINC CONSULTANCY LLC (dba Military Disability Nexus), registered in Sheridan, Wyoming. (militarydisabilitynexus) Six services: Independent Medical Opinion/Nexus Letter, Claim Readiness Review, DBQ (MilitaryDisabilityNexus) (starting at \$249, 5–7 business days), (militarydisabilitynexus) Aid & Attendance, 1151 Claims, C&P Exam Coaching. (MilitaryDisabilityNexus) Claimed turnaround 7–10 business days, 1,000+ veterans served, 98% satisfaction. Clinicians described as MD/DO/NP. (militarydisabilitynexus) (MilitaryDisabilityNexus) Explicit, well-handled disclaimers: not VA-affiliated, not a VSO/claims agent/attorney, does not file claims or communicate with the VA. (Militarydisabilitynexus) (MilitaryDisabilityNexus)

Strengths that give MDN an unfair advantage if amplified. The **scope-discipline positioning is better than most competitors** — it preempts the accreditation-issue flags that hurt Trajector and VA Claims Insider in AI trust evaluations. Dedicated **Editorial Policy and Medical Review Policy** pages as separate documents is a sophisticated YMYL trust architecture almost no peer has. Leaning into **causation mechanisms** (obesity as intermediate step, (Militarydisabilitynexus) (Militarydisabilitynexus) gut-brain axis for Crohn's/PTSD) (militarydisabilitynexus) is more medically substantive than fill-in-the-blank nexus peers. **Transparent DBQ pricing** at \$249 is genuinely differentiated — most competitors obscure pricing entirely. Refusing DBQs that require in-person exams (Militarydisabilitynexus) is an ethical guardrail that reads as honest.

The catastrophic E-E-A-T gap: no named clinicians anywhere. The homepage says "MD/DO/NP"; the About page says "Subject Matter Expert and/or Licensed Clinicians" (Militarydisabilitynexus) — but **no individual bios, photos, credentials, state license numbers, or LinkedIn profiles are published.** Every credible competitor (Prestige Veteran's Leah Bucholz, PA-C; (Prestige Veteran Medical C...) VetNexusMD's Dr. Lee; Telemedica's panel; Hill & Ponton's attorneys; CCK's partners) names and credentials their experts prominently. This is the single largest fixable weakness and the single largest AI-citation unlock — Physician schema with (hasCredential) and (sameAs) pointing to state medical board lookups is what 2025–2026 AI models actually parse to determine YMYL trust. (Redot Global)

Content inventory is thin. Only ~5 confirmed blog posts (all good, all in the nexus/secondary lane: claim readiness, aggravation of pre-existing conditions, obesity as intermediate cause, Crohn's secondary to PTSD, 1151 intro) and 3 case studies (knee aggravation + asthma, OSA secondary to PTSD via obesity, IBS secondary to GAD). (militarydisabilitynexus) No blog tags or articles for PACT Act, TDIU, SMC, MST, TBI, Gulf War, burn pits, Camp Lejeune, tinnitus, migraines, primary PTSD, HLR/BVA, CUE, effective dates, or buddy statements.

The community forum is effectively nonexistent. The (/community) URL exists but shows no threads, categories, moderators, or user activity. Compare to HadIt.com (26,000+ members) and

VetsBenefits.net (39,000+ topics). Either activate it aggressively or remove it — an empty forum is a negative E-E-A-T signal.

Trust-claim credibility gaps. "1,000+ veterans served" and "98% satisfaction" have no methodology disclosed next to the site's own admission it is new. Wyoming registered-agent address (30 N Gould St Ste R, Sheridan) is a generic LLC formation address, not a clinical office — problematic for YMYL searchers who look up addresses. Only DBQ pricing is published; Nexus, A&A, 1151, Claim Readiness Review have no price transparency. No visible third-party reviews (Google, BBB, Trustpilot).

Content gap matrix (coverage confirmed ✓, mentioned only ●, absent ✕): Nexus letters ✓; DBQs ✓; secondary claims ✓ (strong); Aid & Attendance ✓ (service page only); 1151 ✓ (service + intro); pre-existing aggravation ✓; sleep apnea ✓; C&P exams ● (coaching service, no education); PTSD ●; back/knee ●. **Absent entirely: PACT Act, TDIU, SMC, HLR/Supplemental/BVA appeals, MST, TBI, Gulf War Illness, burn pits, Camp Lejeune, tinnitus, migraines, buddy statements, C-file requests, effective dates, CUE, deferrals, protected ratings, Nehmer retroactivity, PCAFC caregiver issues.** Every ✕ is a top-50 keyword cluster.

5. GEO/AEO strategy for AI search citation

AI engines cite MDN-style content when six conditions hold: **(a)** answer-first lead paragraphs, **(b)** named-clinician bylines with Physician schema, **(c)** verbatim regulatory quotes from 38 CFR and M21-1, **(d)** statistics with citations every 150–200 words, **(e)** FAQPage schema with extractable 50–80 word answers, and **(f)** recency signals (content under 13 weeks old is cited at meaningfully higher rates per Previsible 2025 research). The Princeton/Georgia Tech/Allen AI GEO study (KDD 2024) measured citation lift of **+30–40% from authoritative citations, +37% from quotations, +22–41% from statistics** — with the Law/Government vertical specifically rewarding statistics. Separately, Semrush's AI Visibility Index (100M+ citations) found that **40–60% of cited sources rotate monthly** — GEO is not set-and-forget.

Platform-specific citation logic. ChatGPT cites Wikipedia at ~47.9% of top-10 share, leans heavily on commercial .com content, averages ~7.9 citations per answer, and uses the Bing index (so Bing Webmaster Tools verification is critical). Perplexity draws **46.7% of top-10 citations from Reddit, 13.9% from YouTube**, averages ~21.9 citations per answer, and has strong recency bias — making community forums and clinician-voice YouTube videos disproportionately high-ROI. Google AI Overviews mirrors top-10 SERP ranking (~46% citation overlap with top 10) and now appears in **~48.75% of healthcare-related searches**, but triggers a 61% CTR drop on cited queries — meaning the citation itself is the win, not the click. Gemini over-indexes on Google properties (YouTube, Maps) and barely cites Reddit (0.1%). Claude is conservative, formal, and rewards appropriate hedging — valuable for YMYL compliance.

The universal AI-citable article template. Lead every page with a named clinician byline and "Medically reviewed" date. Open with a **40–60 word direct answer** that stands alone and names the diagnostic code, regulation, and rating levels. Follow with a Key Facts extractable bullet block

(5 items maximum). Use **question-form H2 headings** that mirror People Also Ask and ChatGPT/Perplexity prompt patterns. Quote 38 CFR verbatim in `<blockquote cite="...">` tags. Include a rating table with `<caption>` and `<th scope>`. Insert a statistic every 150–200 words with inline citation. Close with 8–12 FAQ question-answers, each 50–80 words and each schema-marked. Every page ends with a "When to seek accredited help" paragraph linking to va.gov/ogc/apps/accreditation/ — this hedging is an AI trust booster, not a liability. Target 1,500–2,500 words; extractability matters more than length.

Schema markup priority stack. Site-wide: Organization + MedicalBusiness with `[disclaimer]`, `[knowsAbout]`, `[sameAs]` (BBB, LinkedIn, Facebook, YouTube). Clinician pages: Physician with `[hasCredential]` (state license number, board certifications) and `[sameAs]` to state medical board lookups and LinkedIn — this is the highest-leverage E-E-A-T signal. Every article: Article + MedicalWebPage hybrid with `[author]`, `[reviewedBy]`, `[lastReviewed]`, `[citation]` (linking to eCFR and VA.gov), and `[about]` (MedicalCondition with ICD-10 code). Every service page: Service schema with transparent `[offers.price]`. Every condition page: FAQPage with 4+ visible Q&As (FAQPage markup delivers 3.1× higher answer extraction and 3.2× higher AI Overview appearance rate). BreadcrumbList everywhere. Use stable `[@id]` URIs so the Person entity builds authority across all articles.

Crawlability is the #1 AEO failure mode. Verify `[robots.txt]` explicitly allows `[GPTBot]`, `[ClaudeBot]`, `[PerplexityBot]`, `[Google-Extended]`, `[CCBot]`, `[OAI-SearchBot]`, `[ChatGPT-User]`, `[PerplexityUser]`, and `[anthropic-ai]`. Check Cloudflare's "Block AI Scrapers" toggle is off for public content. Ensure server-side rendering (MDN's Next.js build may be JS-heavy — Claude and Perplexity don't execute JS reliably). Submit sitemaps to Bing Webmaster Tools (underrated — ChatGPT runs on the Bing index). Publish `[/llms.txt]` (low priority; major AI crawlers don't yet fetch it per Semrush's 2025 audit, but low-cost insurance).

E-E-A-T action items specific to a non-accredited service. Publish every clinician's name, photo, state license number and state, board certification with issuing body, years practicing, relevant military/VA experience, and publication list. Add "Medically reviewed by" separately from author on every page. Publish a Methodology page (exactly how clinicians review records and apply "at least as likely as not"), Corrections Policy, and keep the existing Editorial and Medical Review Policies. Pitch clinician-quoted articles to Military Times, Task & Purpose, Stars and Stripes, Military.com, The War Horse — ChatGPT over-indexes on news and one Task & Purpose citation is worth 50 backlinks. Avoid any language implying guaranteed rating outcomes, filing assistance, or claims consulting — those trigger 38 USC § 5902/5904 accreditation issues and AI trust flags.

Measurement cadence. Build a 30-prompt test set ("nexus letter for sleep apnea," "what is a DBQ," "at least as likely as not standard," "sleep apnea secondary to PTSD VA rating," etc.). Run monthly across ChatGPT, Perplexity, Gemini, Google AI Mode, Google AI Overviews, Claude (web). Log: cited Y/N, position, URL cited, brand-mention accuracy, competitor cited. Use Profound, Peec AI, Otterly.AI, or Semrush AI Toolkit for automation. Track citation share, brand mention accuracy, AI-referred traffic (GA4 referrer segment), and citation decay rate after 90 days without update.

6. Prioritized content recommendations — 30 pieces with outlines

Each piece follows the AI-citable template (named clinician byline, 40–60 word lead answer, question-form H2s, 38 CFR verbatim quotes, FAQ schema, 1,500–2,500 words, with a statistic every 150–200 words).

Tier 1 — Launch in first 60 days (highest AI-citation ROI)

1. **"What is a Section 1151 claim? A complete medical-evidence guide."** Keywords: Section 1151 claim, VA medical malpractice claim, 38 USC 1151 benefits. Outline: definition (40-word lead); 38 USC § 1151 verbatim; "but for" and proximate cause standards; difference from FTCA; the four evidence elements (pre-treatment baseline, adverse event, additional disability, causation); medical records VA requires; when to seek accredited representation. **Pain points:** #21 opacity of 1151 claims. **Authority signals:** quote 38 USC § 1151 and 38 CFR § 3.361 verbatim; cite specific BVA decisions by number; named clinician byline. **Differentiator:** this is a nearly empty SERP — 3,000-word pillar + 5 spokes = category leadership in 6 months.
2. **"1151 vs. FTCA — which path applies to VA medical errors?"** Keywords: 1151 vs FTCA, VA negligence compensation. Comparison table of standards, timelines, exclusivity, recoverable damages, attorney requirements. **Pain point:** #21.
3. **"Sleep apnea secondary to PTSD — the medical evidence VA requires."** Keywords: sleep apnea secondary to PTSD, can sleep apnea be secondary to PTSD, nexus letter for sleep apnea. Outline: 40-word lead with DC 6847 and 38 CFR § 3.310 citation; peer-reviewed OSA-PTSD literature (Mysliwiec, Yesavage — cite by name); rating criteria table (0/30/50/100%); aggravation vs. causation standard; what a nexus letter must address; proposed 2025–2026 rating-schedule changes. **Pain point:** #5, highest-ROI secondary chain.
4. **"What happens at a VA C&P exam — a clinician's walkthrough."** Keywords: what happens at a C&P exam; C&P exam preparation. Outline: the DBQ the examiner is using; typical duration; the six things a valid exam must document (Correia, Sharp, functional loss, frequency/severity, nexus opinion, C-file review); what "pencil-whipped" looks like; how to request a reschedule; ACE/chart-review option. **Pain points:** #1, #2, #3.
5. **"How to recognize and challenge an inadequate C&P exam — the MFR playbook."** Keywords: inadequate C&P exam, Memorandum for Record VA, VA Form 21-4138 C&P rebuttal. Content: the legal standard for inadequacy (Barr v. Nicholson), what to document post-exam, sample MFR structure, when IMOs/nexus letters rebut exam findings. **Pain point:** #4 — major gap online.
6. **"Nexus letter cost, quality, and what a strong one actually contains."** Keywords: nexus letter cost, what makes a strong nexus letter, how to get a nexus letter. Price transparency table of market ranges (\$500–\$3,000); the 8 elements of a strong IMO per VA law (credentials stated, records reviewed listed, "at least as likely as not" language, rationale tied to medical literature, etc.); red flags of nexus mills. **Pain point:** #9 — direct commercial pipeline.

7. "VA rating for tinnitus — the 10% cap, the 2026 rule changes, and stacking strategies."

Keywords: VA rating for tinnitus, new tinnitus rating criteria. Outline: DC 6260 verbatim; why 10% is the cap; proposed VASRD changes; how tinnitus pairs with hearing loss, migraines, Meniere's, depression, anxiety for effective compensation. **Pain point:** #6 — major gap on stacking strategy.

8. "Migraines secondary to tinnitus and TBI — medical evidence for DC 8100."

Keywords: migraines secondary to tinnitus, migraines secondary to PTSD, VA rating for migraines. Outline: DC 8100 "prostrating attacks" definition; how clinicians document frequency/severity; the "severe economic inadaptability" standard for 50%. **Pain point:** #11 secondary chain + migraines underrated.

9. "PTSD rating criteria — why 70% vs. 50% comes down to the record."

Keywords: VA rating for PTSD, what is the VA rating for PTSD, PTSD 70 vs 50. Outline: 38 CFR § 4.130 verbatim; the General Rating Formula levels; occupational/social impairment framework; what evidence moves a rating from 50% to 70% to 100%; role of collateral statements. **Pain point:** #7.

10. "PACT Act presumptive conditions — the 2026 list and what still gets denied."

Keywords: PACT Act presumptive conditions, PACT Act 2026 new conditions, burn pit VA claim. Full conditions table; qualifying service locations/dates; how to reopen previously-denied claims; why some PACT claims still fail. **Pain point:** #10.

Tier 2 — Launch in days 60-120 (fill the biggest topical gaps)

11. "TDIU and extraschedular TDIU under 38 CFR 4.16(b) — when you qualify."

Keywords: TDIU requirements, how to apply for TDIU, TDIU vs 100 schedular. The 60%/70% threshold; Form 21-8940 and 21-4192; "substantially gainful" vs. marginal employment vs. sheltered; passive income rules; extraschedular pathway. **Pain point:** #11 — major gap.

12. "SMC — the compensation most 100% veterans are missing."

Keywords: SMC VA, SMC-K rate 2026, SMC-S housebound. SMC-K through T explainer; bilateral factor; stacking SMC-Ks. **Pain point:** #26 — major gap.

13. "Aid & Attendance eligibility after the 2018 asset rules — what qualifies now."

Keywords: Aid and Attendance eligibility. Net-worth limit, medical-expense deductions, three-year look-back; Housebound vs. A&A distinction; Form 21-2680 clinician documentation requirements. **Pain point:** #22.

14. "Higher Level Review, Supplemental Claim, or BVA — which appeal lane matches your case."

Keywords: Higher Level Review VA, Supplemental Claim VA, how to appeal VA disability denial. Decision tree; HLR no-new-evidence rule; Supplemental "new and relevant" standard; BVA three dockets. **Pain point:** #12.

15. "MST claims — what 'markers' are, and how clinicians document them."

Keywords: VA disability for MST, MST PTSD VA claim. STR indicators (transfers, pregnancy/STI tests, behavioral changes); specialized MST claim process; clinician role in corroborating through current-symptom presentation. Handle with trauma-informed language. **Pain point:** #20 — major gap.

16. "How to request and read your C-file — the mechanics of VA Form 20-10206." Keywords: how to request C-file, VBMS C-file. Step-by-step; timelines; how to read rater notes and exam results. **Pain point:** #19 — major gap.

17. "Effective dates and Nehmer retroactivity — the back pay many veterans are owed." Keywords: effective date VA claim, VA disability back pay. Intent-to-file rules; within-one-year-of-discharge rule; Nehmer and PACT liberalizing-law dates. **Pain point:** #13 — major gap.

18. "Buddy statements — who can write one, what to include, and why they matter." Keywords: buddy statement VA, lay evidence VA, VA Form 21-10210. Eligibility; content structure; PTSD-stressor and MST use cases. **Pain point:** #18.

19. "Protected ratings explained — the 5-, 10-, and 20-year rules." Keywords: protected rating VA, VA rating reduction. Stabilization vs. service-connection vs. continuous-rating protections; P&T status. **Pain point:** #17 — major gap.

20. "CUE claims — when undebatable error is the right tool, and when it isn't." Keywords: CUE claim VA. The three-prong standard; why CUE fails for duty-to-assist violations; when HLR or supplemental beats CUE. **Pain point:** #24 — major gap.

21. "Camp Lejeune — the VA claim and the Justice Act lawsuit are two separate tracks." Keywords: Camp Lejeune VA disability. Presumptive conditions; service-period window (1953–1987); how a CLJA lawsuit and a VA claim interact. **Pain point:** #27.

22. "Gulf War Illness and MUCMI — the undiagnosed-illness presumption." Keywords: Gulf War illness VA claim, MUCMI VA. Undiagnosed illness vs. diagnosed chronic multisymptom illness; qualifying service periods; medical documentation challenges.

23. "TBI residuals — the overlap with PTSD, migraines, and cognitive impairment." Keywords: VA rating for TBI, TBI secondary conditions. DC 8045; the 10 facets of cognitive impairment; TBI + PTSD rating analysis.

24. "Hypertension secondary to sleep apnea and PTSD — the causation evidence." Keywords: hypertension secondary to sleep apnea, hypertension secondary to PTSD. Peer-reviewed linkage literature; PACT Act Agent Orange addition; Nehmer retroactivity for Vietnam-era claimants.

25. "Radiculopathy secondary to back and neck conditions." Keywords: radiculopathy secondary to back pain, VA rating for radiculopathy. DC 8520 et seq.; mild/moderate/severe distinction; EMG evidence.

Tier 3 — Launch in days 120–180 (depth and distinctiveness)

26. "The 2026 VASRD rating-schedule changes — sleep apnea, tinnitus, mental health." Keywords: 2026 VA rating schedule changes, new sleep apnea rating rules. Living, quarterly-updated page tracking Federal Register rulemaking. Very AI-citable, highly time-sensitive.

27. "PCAFC caregiver denials and tier reductions — the 2020 expansion fallout." Keywords: PCAFC caregiver denial, MISSION Act caregiver. **Pain point:** #23 — major gap.

28. "GERD and IBS secondary to PTSD — the gut-brain axis evidence." Keywords: GERD secondary to PTSD, IBS secondary to PTSD. Builds on MDN's existing Crohn's post.

29. "Erectile dysfunction, PTSD, and SMC-K — the claim most veterans don't file." Keywords: erectile dysfunction secondary to PTSD. SSRI causation; SMC-K qualification; confidentiality framing.

30. "How to choose between free VSOs, paid services, and VA-accredited attorneys." Keywords: VSO vs attorney VA, VA disability attorney cost. Honest comparison — free (DAV/VFW/AL), paid coaching (regulatory gray zone), accredited agents, attorneys with 20%/33% fees. Includes what MDN does and doesn't do. Highest-integrity content on the site and a key trust signal. **Pain point:** #25.

7. Forum thread starter ideas — seed the community to earn Perplexity citations

Perplexity draws 46.7% of top-10 citations from Reddit and 31% from social/community sources. A branded forum with clinician-moderated answers is MDN's single highest-leverage UGC bet. Seed with 25 starter threads, each with a clinician-authored top-level answer (Q&A page schema).

Diagnostic and claim strategy threads. "What did your 'inadequate C&P exam' look like — and what did you do about it?"; "Share your nexus letter experience — cost, turnaround, and did it work?"; "Veterans who filed 1151 claims — what evidence moved the needle?"; "Sleep apnea secondary to PTSD — what did your approved nexus letter actually say?"; "Which secondary condition chains are most underused?"

Evidence and documentation threads. "What STR entries turned out to be critical for your claim?"; "How did you find buddy statement authors after losing contact?"; "MST claims — what markers did your rater accept?"; "How long did your C-file FOIA request actually take?"; "What does a 'pencil-whipped' DBQ look like and how did you prove it?"

Appeals and procedural threads. "HLR, Supplemental, or BVA — which did you choose and why?"; "Anyone pursued extraschedular TDIU under 38 CFR 4.16(b)?"; "CUE motions — successes and failures"; "Effective date disputes — the Nehmer/PACT back pay stories"; "Rating reduction after an increase claim — did it happen to you?"

Condition-specific threads. "Migraines rated at 50% — what did your claim file contain?"; "Tinnitus and hearing-loss combos — what rating math worked?"; "TBI residuals — how did your claim separate TBI from PTSD?"; "Burn pit claims — which conditions are getting approved in 2026?"; "Camp Lejeune — filed the VA claim, the CLJA lawsuit, or both?"

Industry and process threads. "VSO experiences — the good, the bad, and the ghosting"; "Paid services — which coaches helped and which felt predatory?"; "C&P contractors — QTC vs. LHI vs. VES experiences"; "VA.gov portal dysfunction — workarounds that actually work"; "SMC-K, bilateral factor, and combinations — what most vets miss."

Each thread should be pinned with a 200–300 word clinician-authored primer citing 38 CFR, launched with 3–5 authentic veteran responses (either recruited or from consented case-study subjects), and marked up with Q&APage schema.

8. Implementation roadmap

Days 1–30 — foundation and crawlability. Verify `robots.txt` allows all major AI bots and Cloudflare's AI-scraper block is off. Confirm server-side rendering on all content pages. Verify Bing Webmaster Tools. Deploy Organization + MedicalBusiness schema site-wide with `disclaimer`, `knowsAbout`, and `sameAs`. Publish named clinician bios with photos, state license numbers, board certifications, and Physician schema including `hasCredential` and `sameAs` to state medical board lookups — **this single change is the largest E-E-A-T unlock available**. Add author bylines and `reviewedBy` to every existing blog post and case study. Publish transparent pricing on all six service pages. Replace or clarify the Wyoming registered-agent address. Either seed `/community` with 10 starter threads or remove it from navigation.

Days 31–90 — Tier 1 content launch. Publish the 10 Tier 1 articles using the AI-citable template. Launch the **Section 1151 pillar + 5 spokes** (highest-ROI SERP territory). Deploy FAQPage schema on every service and condition page. Build a "Conditions by Diagnostic Code" hub page. Build one interactive tool (Nexus Strength Scorer or Claim Readiness Checklist) — Hill & Ponton's calculator earns steady citations by being the canonical tool. Stand up a 30-prompt monthly AI-citation audit across ChatGPT, Perplexity, Gemini, Google AI Overviews, and Claude.

Days 91–180 — Tier 2 content and authority building. Publish Tier 2's 15 articles filling the PACT/TDIU/SMC/appeals/MST/TBI/C-file/effective-date gaps. Pitch clinician-quoted pieces to Military Times, Task & Purpose, Stars and Stripes, Military.com, The War Horse — aim for 3 placements. Launch clinician-voice YouTube content for Perplexity and Gemini citation. Publish Methodology and Corrections Policy pages. Integrate third-party reviews (BBB, Google Reviews, Trustpilot). Activate forum with 25 seeded threads.

Days 181–365 — depth, freshness, and measurement. Publish Tier 3's 5 articles. Refresh all Tier 1 content quarterly — 50% of AI-cited content is under 13 weeks old. Expand secondary-condition-chain coverage to cover all 10 K-cluster keywords. Build a living "2026 VASRD changes" tracker page updated with each Federal Register notice. Escalate Bing Webmaster Tools optimization. Monitor citation share monthly and rotate underperforming prompt targets.

What to measure. Citation share (% of the 30-prompt test set where MDN is cited) is the north-star KPI. Brand-mention accuracy (does the AI correctly reproduce the scope disclaimer?) is the secondary KPI. AI-referred traffic in GA4 (segment by referrer: chat.openai.com, perplexity.ai, claude.ai, gemini.google.com, chatgpt.com) is the business-impact KPI. Server-log AI-bot crawl frequency (GPTBot, ClaudeBot, PerplexityBot, Google-Extended) is the leading indicator.

Conclusion — the win is precision, not volume

The temptation with a VA-disability content strategy is to publish 200 articles and compete on volume. The evidence says the opposite: MDN wins by being the most **clinically rigorous, regulatorily literal, transparently scoped, and named-clinician-credentialed** source in the medical-documentation category. Every competitor of scale either lacks named clinician authorship (Trajector, REE, Valor 4 Vet), lacks regulatory precision (VA Claims Insider), or has accreditation-issue baggage that AI models have learned to downweight (the VA warning-letter cohort). MDN can own the "What medical evidence does the VA require for [condition]?" and "What goes in a nexus letter for [condition]?" query clusters — the exact query space where Perplexity, ChatGPT, Claude, and AI Overviews are actively searching for credible, non-conflicted medical authorities. The Section 1151 cluster is a near-empty SERP waiting for a category leader, and MDN already has the service to anchor it. Pair that with activated community UGC seeded by clinicians, and MDN has a realistic path to becoming a default-cited source within 12 months. The constraint is not strategy — it is execution discipline on the template, the schema, the bylines, and the quarterly refresh cadence.